

# Horizon Care Network

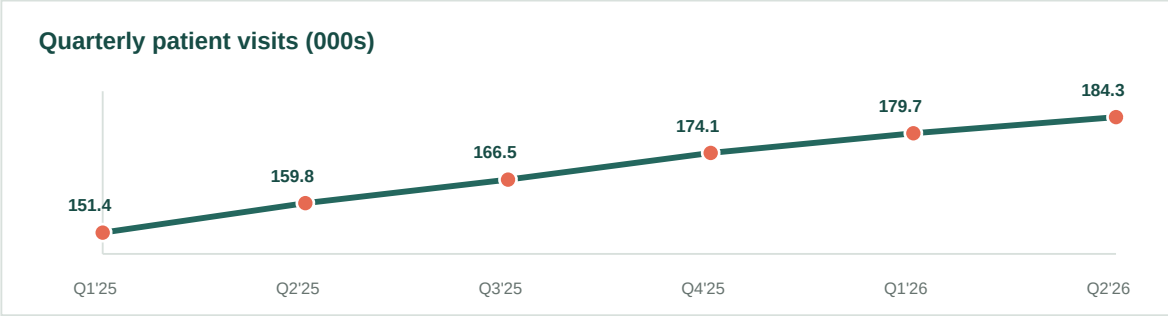
## Q2 2026 Operating Review

Multi-site outpatient care, clinical quality, patient access, workforce, and financial performance

|                     |                                       |              |                                     |
|---------------------|---------------------------------------|--------------|-------------------------------------|
| Reporting Period    | Q2 2026                               | Report Type  | Quarterly Operating Review          |
| Currency / Units    | USD, millions unless noted            | Submitted By | Elena Ruiz, Chief Financial Officer |
| Operating Footprint | 31 outpatient centres across 5 states | Data Through | June 30, 2026                       |

|                          |         |                |
|--------------------------|---------|----------------|
| NET PATIENT REVENUE      | \$42.7M | +11.8% YoY     |
| ADJUSTED EBITDA          | \$6.4M  | 15.0% margin   |
| PATIENT VISITS           | 184,320 | +9.6% YoY      |
| SAME-CENTRE VISIT GROWTH | 5.4%    | Year over year |
| CASH BALANCE             | \$18.9M | June 30, 2026  |
| TOTAL EMPLOYEES          | 1,146   | 742 clinical   |

**Quarter at a glance.** Horizon delivered record quarterly visits and revenue, supported by five recently opened centres, improved provider utilization, and stronger commercial-payer volumes. Adjusted EBITDA exceeded plan by \$0.6M. Patient access improved, but staffing pressure in two metropolitan markets continued to affect wait times and weekend coverage.



This document is a fictional healthcare-services reporting package created for software demonstration, PDF extraction testing, and human-validation workflow design.

# 1. Executive and Board Summary

Horizon Care Network operates urgent care, primary care, and employer-health clinics in five states. The network ended Q2 2026 with 31 operating centres, including five sites opened during the past twelve months. Management's focus remains balanced growth, clinical quality, patient access, and disciplined site-level economics.

| Theme            | Q2 2026 outcome                                                               | Management interpretation                                                     |
|------------------|-------------------------------------------------------------------------------|-------------------------------------------------------------------------------|
| Growth           | Net patient revenue of \$42.7M, 3.4% above budget; visits increased 9.6% YoY. | Volume growth came from new centres and higher same-centre demand.            |
| Profitability    | Adjusted EBITDA of \$6.4M and margin of 15.0%.                                | Provider productivity and centralized scheduling offset wage pressure.        |
| Clinical quality | Patient satisfaction 91 NPS; 7-day return rate 4.2%.                          | Quality indicators remain within policy; two sites require targeted coaching. |
| Access           | Median door-to-provider time was 19 minutes versus 22 minutes in Q1.          | Digital check-in and staffing changes improved throughput.                    |
| Risk             | Nurse-practitioner vacancies remain elevated in Phoenix and Denver.           | Contract coverage protects schedules but increases labour cost.               |

## Board decisions and management follow-ups

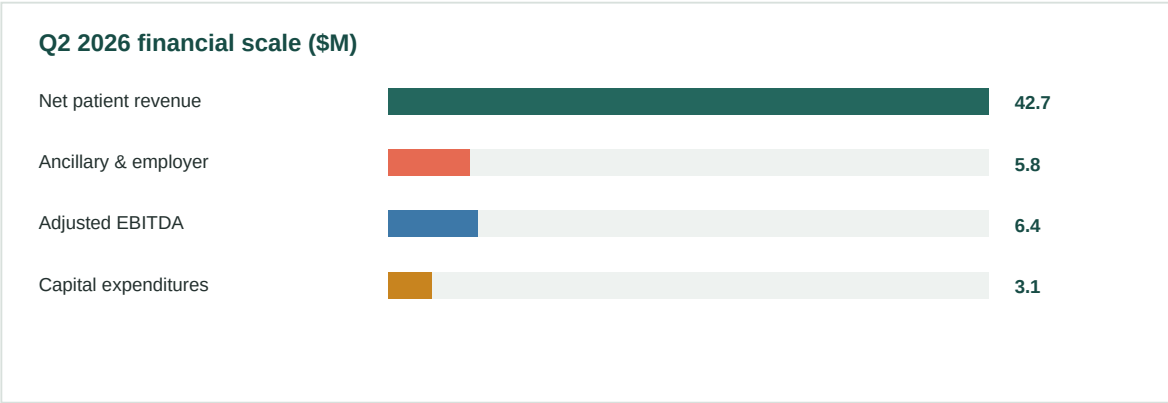
| Priority | Decision / follow-up                                                        | Owner             | Due date |
|----------|-----------------------------------------------------------------------------|-------------------|----------|
| High     | Approve two new centres for Austin and Tampa, subject to final lease terms. | CEO / Development | Jul 24   |
| High     | Approve revised clinical compensation model for hard-to-staff markets.      | COO / CMO         | Aug 15   |
| Medium   | Complete revenue-cycle vendor review and denial-reduction plan.             | CFO               | Sep 1    |
| Medium   | Review employer-health acquisition target and integration requirements.     | Corp Dev / COO    | Sep 20   |

**Outlook.** Management expects FY2026 net patient revenue of \$169.0M to \$171.0M and adjusted EBITDA of \$24.0M to \$25.0M. The outlook assumes stable payer mix, continued provider hiring, and no material reimbursement changes.

## 2. Financial Performance

Q2 2026 Actual represents the three months ended June 30, 2026. Current-period results are shown beside prior-quarter, prior-year, and budget values. Forecasts are clearly labelled and should not be treated as current-quarter actuals.

| Metric                       | Q2 2026 Actual | Q1 2026 Actual | Q2 2025 Actual | Q2 2026 Budget | FY2026 Forecast |
|------------------------------|----------------|----------------|----------------|----------------|-----------------|
| Net Patient Revenue          | \$42.7M        | \$40.1M        | \$38.2M        | \$41.3M        | \$170.2M        |
| Ancillary & Employer Revenue | \$5.8M         | \$5.4M         | \$4.9M         | \$5.6M         | \$23.5M         |
| Total Revenue                | \$48.5M        | \$45.5M        | \$43.1M        | \$46.9M        | \$193.7M        |
| Clinical Labour Expense      | \$17.6M        | \$16.9M        | \$15.8M        | \$17.2M        | \$70.8M         |
| Adjusted EBITDA              | \$6.4M         | \$5.5M         | \$4.9M         | \$5.8M         | \$24.6M         |
| Adjusted EBITDA Margin       | 13.2%          | 12.1%          | 11.4%          | 12.4%          | 12.7%           |
| Capital Expenditures         | \$3.1M         | \$4.2M         | \$2.4M         | \$3.5M         | \$13.2M         |
| Cash and Cash Equivalents    | \$18.9M        | \$17.4M        | \$14.6M        | \$16.8M        | \$21.5M         |
| Total Debt                   | \$42.0M        | \$43.5M        | \$45.0M        | \$42.8M        | \$39.0M         |



**Financial commentary.** Total revenue exceeded budget by \$1.6M. Adjusted EBITDA was \$0.6M above plan due to stronger visit volume, improved provider productivity, and lower-than-planned temporary clinical labour. Cash increased by \$1.5M from Q1 after debt service and capital spending.

### 3. Clinical Quality and Patient Access

| Metric                          | Q2 2026    | Q1 2026    | Q2 2025    | Operating target |
|---------------------------------|------------|------------|------------|------------------|
| Patient Visits                  | 184,320    | 179,740    | 168,180    | 180,000+         |
| Same-centre Visit Growth        | 5.4%       | 4.9%       | 3.8%       | At least 4.0%    |
| Patient Satisfaction (NPS)      | 91         | 89         | 87         | At least 88      |
| Median Door-to-Provider Time    | 19 minutes | 22 minutes | 24 minutes | Below 20 minutes |
| 7-day Return Rate               | 4.2%       | 4.4%       | 4.6%       | Below 5.0%       |
| Telehealth Visits               | 21,480     | 18,960     | 14,220     | Grow selectively |
| No-show Rate                    | 6.8%       | 7.4%       | 7.9%       | Below 7.0%       |
| Average Daily Visits per Centre | 65.3       | 64.7       | 61.9       | At least 64.0    |

#### Quality and compliance events

| Area                             | Q2 result            | Commentary                                                    |
|----------------------------------|----------------------|---------------------------------------------------------------|
| Serious safety events            | 0                    | No reportable sentinel events during the quarter.             |
| Medication-related incident rate | 0.7 per 1,000 visits | Within policy threshold; training refreshed at three sites.   |
| Regulatory inspections           | 4 completed          | No material findings; two minor documentation actions closed. |
| Patient complaints               | 1.8 per 1,000 visits | Primary themes were wait time and billing clarity.            |

**Historical context:** Q4 2025 patient satisfaction was 88 and the annual 2025 no-show rate was 7.6%. These figures are included for context and should not replace the current Q2 2026 NPS of 91 or no-show rate of 6.8%.

4. Payer Mix and Revenue Cycle

| Payer category       | Q2 2026 share | Q1 2026 share | Q2 2025 share | Average days to collect |
|----------------------|---------------|---------------|---------------|-------------------------|
| Commercial Insurance | 46%           | 45%           | 44%           | 27 days                 |
| Medicare             | 21%           | 22%           | 22%           | 32 days                 |
| Medicaid             | 17%           | 17%           | 18%           | 39 days                 |
| Employer Contracts   | 9%            | 9%            | 8%            | 24 days                 |
| Self-pay             | 7%            | 7%            | 8%            | 18 days                 |

Revenue-cycle performance

| Metric                      | Q2 2026   | Q1 2026   | Q2 2025   | Target         |
|-----------------------------|-----------|-----------|-----------|----------------|
| Days in Accounts Receivable | 31.4 days | 33.2 days | 36.0 days | Below 32 days  |
| Initial Claim Denial Rate   | 5.8%      | 6.4%      | 7.1%      | Below 6.0%     |
| Net Collection Rate         | 97.1%     | 96.6%     | 95.8%     | At least 97.0% |
| Bad Debt as % of Revenue    | 1.9%      | 2.1%      | 2.4%      | Below 2.2%     |
| Unbilled Revenue            | \$2.6M    | \$2.8M    | \$3.1M    | Monitor        |

**Revenue-cycle update.** Denials and days in accounts receivable improved after claim-scrubbing automation and centralized follow-up. Medicaid collections remain slower than other payer categories. Management is evaluating two revenue-cycle vendors for 2027.

5. Workforce and Centre Performance

| Metric                         | Q2 2026 | Q1 2026 | Q2 2025 | FY2026 plan |
|--------------------------------|---------|---------|---------|-------------|
| Total Employees                | 1,146   | 1,102   | 986     | 1,210       |
| Clinical Employees             | 742     | 713     | 638     | 780         |
| Physicians                     | 126     | 121     | 108     | 132         |
| Nurse Practitioners / PAs      | 214     | 201     | 176     | 228         |
| Medical Assistants             | 286     | 277     | 252     | 300         |
| Corporate & Shared Services    | 118     | 115     | 102     | 122         |
| Annualized Voluntary Attrition | 14.2%   | 15.0%   | 17.6%   | Below 15.0% |
| Open Clinical Roles            | 47      | 54      | 61      | Below 45    |

Selected centre performance

| Centre                 | Market      | Q2 visits | Revenue | EBITDA margin | Access / issue                  |
|------------------------|-------------|-----------|---------|---------------|---------------------------------|
| Horizon Austin Central | Austin, TX  | 10,840    | \$2.8M  | 19.2%         | Median wait 16 minutes          |
| Horizon Phoenix North  | Phoenix, AZ | 9,120     | \$2.2M  | 9.8%          | NP vacancies affecting weekends |
| Horizon Denver Tech    | Denver, CO  | 8,760     | \$2.1M  | 11.1%         | Temporary labour above plan     |
| Horizon Tampa Bay      | Tampa, FL   | 9,940     | \$2.5M  | 17.6%         | Employer-health volume ahead    |
| Horizon Raleigh West   | Raleigh, NC | 7,880     | \$1.9M  | 15.9%         | Patient satisfaction NPS 94     |

**Workforce note.** The network added 44 employees during Q2. Hiring remains concentrated in advanced-practice providers, medical assistants, and revenue-cycle roles. Phoenix and Denver use temporary clinical staff while permanent recruiting continues.

## 6. Growth Program, Outlook, and Risks

| Project / initiative     | Market       | Approved spend | Spent to date | Status         | Expected opening / completion |
|--------------------------|--------------|----------------|---------------|----------------|-------------------------------|
| Austin South Centre      | Austin, TX   | \$2.4M         | \$1.7M        | Construction   | Sep 2026                      |
| Tampa East Centre        | Tampa, FL    | \$2.1M         | \$0.9M        | Construction   | Nov 2026                      |
| Phoenix Scheduling Hub   | Phoenix, AZ  | \$0.8M         | \$0.5M        | Implementation | Aug 2026                      |
| Revenue-cycle automation | Network-wide | \$1.3M         | \$0.7M        | Pilot          | Oct 2026                      |
| Employer-health platform | Network-wide | \$1.0M         | \$0.4M        | Development    | Dec 2026                      |

### FY2026 outlook

| Metric               | FY2026 guidance       | Prior guidance        | Key assumption                              |
|----------------------|-----------------------|-----------------------|---------------------------------------------|
| Net Patient Revenue  | \$169.0M - \$171.0M   | \$165.0M - \$168.0M   | Visit growth and stable payer mix           |
| Total Revenue        | \$192.0M - \$195.0M   | \$188.0M - \$192.0M   | Employer-health and ancillary growth        |
| Adjusted EBITDA      | \$24.0M - \$25.0M     | \$22.5M - \$24.0M     | Productivity and revenue-cycle improvement  |
| Capital Expenditures | \$13.2M               | \$13.5M               | Two centre openings and technology projects |
| Year-end Cash        | Approximately \$21.5M | Approximately \$19.0M | No acquisition cash outflow assumed         |

### Key risks and mitigations

| Risk                           | Assessment    | Mitigation                                                                   |
|--------------------------------|---------------|------------------------------------------------------------------------------|
| Clinical staffing availability | High          | Market-based compensation, recruiting partnerships, and flexible scheduling. |
| Reimbursement pressure         | Medium        | Payer-contract review, coding controls, and service-line diversification.    |
| Revenue-cycle execution        | Medium        | Automation pilot, denial analytics, and vendor-accountability measures.      |
| New-centre ramp                | Medium        | Stage openings against hiring readiness and local demand milestones.         |
| Clinical quality variation     | Low to medium | Site scorecards, peer review, and targeted medical-director support.         |

### Management certification

Management confirms that current-period financial, operational, and clinical metrics in this report reflect the quarter ended June 30, 2026, subject to normal quarter-end adjustments. Forecasts and planned openings are forward-looking estimates and should not be treated as current-period actual results.

**Elena Ruiz**

Chief Financial Officer

Horizon Care Network

July 10, 2026